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NOW **A deserted Covid-19 vaccination centre in Daryaganj, Delhi, Oct. 14, 2022**

on immunisation in India decided that the data for these vaccines were not enough to prove a major advantage. "If variants causing more severe disease do come up, then perhaps we will have to look at the value of bivalent vaccines," says Dr Arora. In the interim, the government can also invest in research on a vaccine that could offer broad protection against all coronavirus strains and for a longer duration. "It is unlikely we will have one quickly, but it is a good goal from a scientific perspective," says Dr Kang.

Another area of focus experts recommend is vulnerable populations such as the elderly or those with co-morbidities. They remain at risk for severe disease. In the US, the vulnerable are being given Paxlovid, an antiviral known to work even if a variant with immune escape mutations comes up. "In India," says Dr Kang, "we need a strategy that includes rational protocol and takes into account the latest antibody, anti-inflammatory and antiviral treatments for Covid. Now is the time to build upon the treatment side of things."

There is also growing demand that medical attention be given to Long Covid, symptoms of which studies have shown can last from anywhere between 12 and 18 months. The Long Covid OPD at LNJP, for instance, routinely receives patients complaining of insomnia, weakness, irritability, poorly controlled diabetes, bodyache and psychological trauma. Even a minor Covid illness can leave a person with fatigue that can last for weeks after the infection has left the body. "Many from the first and second wave have still not recovered completely," says Dr Kumar. "You need specialised, not general care to understand their issues and treat them." One solution may be to set up specialised Long Covid centres at every tertiary care hospital so that care for Covid-specific health problems is available even after the pandemic comes to an end.

The war against Covid may seem over in India, but the larger battle is far from won. The makeshift Covid facilities were a commendable effort, but India's health infrastructure continues to be patchy and abysmal. We still have just 0.5 beds per 1,000 persons, with the average dropping to 0.24 in district hospitals. This when WHO guidelines mandate a ratio of 5:1,000. There is also an ongoing shortage of rural doctors, with community health centres reporting a 76 per cent shortfall in specialist doctors.

Covid-19 was a deadly reminder of how a tiny virus, 2,000 times smaller than an ordinary dust mite, could bring the whole world to a standstill. But it also demonstrated the power and resilience of the human spirit, and how it could rally in the face of collective adversity. Vaccines against the virus were developed within a year in several countries, treatment breakthroughs and genomic sequences shared across borders and the medical community in every country came together to make sense of the affliction and ways to overcome it. While the world may have retreated into physical isolation for two years, it was united in suffering. Several countries, including India, are now deploying the new research and learning from Covid to predict, prevent and perhaps tame the next virus. For the moment, though, it is enough to just breathe easy. ■